

Digital Patient Experience and Telehealth Expansion in a Healthcare Network

Consulting Case Study | Digital Transformation, Healthcare

Executive Summary (TLDR)

A regional healthcare network with 18 clinics was struggling with high appointment no-show rates, long phone-based scheduling wait times, and underused telehealth capacity built during a prior initiative. A digital patient experience transformation redesigned scheduling, communication, and telehealth workflows, reducing no-show rates from **22% to 11%**, cutting average scheduling call time from **8 minutes to under 2**, and increasing telehealth utilization from **9% to 34%** of eligible visits within 12 months.

Table of Contents

- Background
- Methodology
- Key Considerations
- Sample Deliverables
- Strategic Recommendations
- Culture and Change Management
- Technology and Platform Architecture
- Patient Journey Redesign
- Regulatory and Privacy Considerations
- Risk Identification
- Cost-Benefit Analysis
- Monitoring and Adoption Metrics
- Integration with Clinical Systems
- Return on Investment
- Establishing a Digital Operating Model
- Change Management Playbook
- Additional Considerations
- Key Findings and FAQs

1. Background

A regional healthcare network operating 18 primary and specialty care clinics had invested in telehealth capability during a prior initiative, but utilization remained low at just 9% of eligible visits — well below the network's own targets and below regional peer benchmarks. Meanwhile, appointment no-show

rates sat at 22%, driven partly by a phone-only scheduling process with average call wait times of eight minutes, which discouraged patients from calling back to reschedule or confirm appointments.

Clinic staff reported that patients frequently didn't know telehealth was an available option for their visit type, and that the scheduling phone line was overwhelmed during peak hours, pushing some patients to simply not show up rather than wait on hold. Leadership needed to determine whether investment should focus on promoting the existing telehealth capability more heavily, or on a broader redesign of the patient scheduling and communication experience that telehealth adoption depended on.

2. Methodology

A **6-phase digital transformation approach** was applied:

- **Phase 1 – Patient Journey Diagnostic:** Map the scheduling, reminder, and visit journey to identify where patients disengaged or defaulted to no-show.
- **Phase 2 – Telehealth Eligibility Analysis:** Determine which visit types were clinically appropriate for telehealth and why eligible patients weren't choosing it.
- **Phase 3 – Digital Scheduling Redesign:** Build an online scheduling option integrated with clinic calendars, reducing dependence on phone-based booking.
- **Phase 4 – Communication Redesign:** Implement automated, multi-channel appointment reminders and telehealth-option prompts at the point of scheduling.
- **Phase 5 – Phased Clinic Rollout:** Pilot the redesigned experience at a subset of clinics before network-wide deployment.
- **Phase 6 – Scale & Optimize:** Expand to all clinics and continuously refine based on no-show and utilization data.

3. Key Considerations

- Low telehealth utilization wasn't primarily an awareness problem — it was a default problem: patients weren't being offered telehealth as an option at the point of scheduling for eligible visit types.
- No-show rates were closely tied to scheduling friction; patients who couldn't easily reschedule when their circumstances changed simply didn't show up rather than navigating a long phone wait.
- Older patient populations required a digital scheduling experience that didn't assume high digital literacy, alongside continued phone support rather than a phone-line replacement.

4. Sample Deliverables

- Patient journey diagnostic with no-show driver analysis
- Telehealth eligibility criteria and default-offer framework
- Online scheduling platform integration plan
- Automated multi-channel reminder and communication workflow
- Clinic-level rollout and adoption tracking dashboard

5. Strategic Recommendations

The recommended approach centered on three changes: (1) implementing online self-scheduling integrated with clinic calendars to reduce dependence on the overwhelmed phone line; (2) defaulting eligible visit types to offer telehealth as an explicit choice at the point of scheduling rather than requiring patients to ask for it; and (3) replacing single-channel appointment reminders with automated text, email, and app notifications including one-tap rescheduling.

6. Culture and Change Management

Clinic front-desk staff were concerned that online scheduling would reduce their role and create confusion if patients booked inappropriate appointment types. The program addressed this by positioning online scheduling as a capacity release valve for phone staff — reducing call volume so staff could spend more time on complex scheduling needs and patient support — with clear escalation paths built into the online system for ambiguous cases.

7. Technology and Platform Architecture

An online scheduling platform was integrated directly with existing clinic calendar systems, allowing real-time appointment booking without manual reconciliation by front-desk staff. Automated communication workflows were layered on top to handle reminders and telehealth prompts based on visit type and patient history.

8. Patient Journey Redesign

The scheduling journey was redesigned so that telehealth-eligible visit types presented telehealth as a default, equally prominent option alongside in-person scheduling, rather than an obscure alternative patients had to specifically request.

9. Regulatory and Privacy Considerations

The telehealth and digital scheduling platforms were implemented in full compliance with applicable healthcare privacy regulations, with particular attention to secure video-visit infrastructure and patient data handling across the newly integrated scheduling and communication systems.

10. Risk Identification

Key risks included the possibility of increased no-shows if online scheduling made booking too frictionless without adequate reminder follow-up, clinical risk if patients selected telehealth for visit types requiring in-person examination, and the risk of alienating less digitally comfortable patients. Each was mitigated through automated reminder sequencing, clinically validated eligibility rules, and continued phone-support availability.

11. Cost-Benefit Analysis

Platform investment was weighed against the value of reduced no-show-related revenue loss, reduced phone-staffing pressure, and better utilization of existing telehealth infrastructure that had already been paid for but underused. The business case showed payback within the first year primarily through no-show reduction.

12. Monitoring and Adoption Metrics

No-show rate, average scheduling call time, telehealth utilization rate, and online scheduling adoption were tracked weekly during the pilot phase, allowing rapid identification of clinics or visit types where the redesigned journey wasn't performing as expected.

13. Integration with Clinical Systems

The scheduling and communication platforms were integrated with the network's electronic health record (EHR) system to ensure visit-type eligibility rules and patient history were reflected accurately in real time, avoiding scheduling errors that could create clinical risk.

14. Return on Investment

Within 12 months, no-show rates fell from 22% to 11%, average scheduling call time dropped from eight minutes to under two, and telehealth utilization rose from 9% to 34% of eligible visits, substantially improving both patient access and clinic operational efficiency.

15. Establishing a Digital Operating Model

A Patient Digital Experience team was established, combining clinical operations, IT, and patient communications representation, to sustain ongoing optimization of scheduling and telehealth adoption beyond the initial rollout.

16. Change Management Playbook

A playbook was developed for front-desk staff transitioning toward higher-value patient support roles, along with a phased clinic rollout sequence and patient communication materials introducing the new scheduling and telehealth options.

17. Additional Considerations

The platform architecture was designed to support future expansion into chronic condition remote monitoring and asynchronous digital visits, building on the scheduling and communication foundation established in this phase.

18. Key Findings and FAQs

Why was telehealth utilization so low despite prior investment? Because eligible visit types weren't defaulting patients toward the telehealth option at the point of scheduling — the capability existed but wasn't being actively offered.

What had the single biggest impact on no-show reduction? Automated, multi-channel reminders with one-tap rescheduling, which let patients easily adjust plans instead of simply not showing up.

How was clinical safety maintained while expanding telehealth defaults? Through clinically validated eligibility rules embedded directly into the scheduling system, ensuring only appropriate visit types were offered telehealth by default.